

THE ALGORITHM

clinIQ GTM Playbook

Healthcare ICP, buyer, trigger, assessment, sales motion, adoption and account-expansion architecture.

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Confidential strategic working document. Verified company information, proposed GTM strategy, personal experience and illustrative planning assumptions should be distinguished when used externally.

1. Product understanding

clinIQ should be understood as an operational layer around the clinic's existing EHR, connecting patient flow, digital check-in, scheduling, analytics, RTM, pre-authorization, telehealth, engagement and related workflows. The commercial conversation should begin with operational and revenue outcomes rather than feature breadth.

Position clinIQ as the operational layer around the EHR: identify where clinic flow, front-desk workload, authorization or eligible revenue processes are creating measurable friction, then prove value in that workflow.

2. Commercial problem

- Manual check-in and front-desk overload.
- Poor visibility into patient location and waiting time.
- Scheduling and room/provider bottlenecks.
- Prior-authorization delays and staff workload.
- RTM opportunities not systematically captured.
- Fragmented messaging, telehealth and patient workflows.

3. Ideal Customer Profile

Priority ICP	Why it matters
Multi-location specialty clinics	Operational complexity and standardization create visible pain.
Behavioral health / addiction	High appointment volume, coordination and recurring engagement workflows.
Physical therapy / orthopedics / pain	Strong scheduling, throughput, authorization and RTM relevance.
Surgery / procedure centres	High-value workflow coordination and prior-authorization dependency.

4. Buying committee

Persona	Primary concern
Practice Owner / CEO	Growth, patient experience, revenue and operating leverage
COO / Practice Administrator	Throughput, wait time, staff productivity and standardization
Revenue Cycle Leader	RTM, pre-authorization, leakage and administrative burden
Clinical Operations	Patient flow and day-to-day usability
IT / EHR Administrator	Integration, security, deployment and support

5. Trigger events

- New clinic/location opening.

- Acquisition or consolidation of practices.
- EHR dissatisfaction without appetite for replacement.
- High wait times or no-show concern.
- RTM programme launch.
- Prior-authorization backlog.
- Rapid provider/patient-volume growth.

6. Positioning and message architecture

Lead with the business outcome: better clinic flow, lower administrative friction and improved capture of relevant revenue workflows without requiring the customer to replace its EHR. Use the strongest pain as the entry point; do not demo all modules at once.

7. Competitive frame

Competitor / category	How to position
Phreesia / intake platforms	Avoid competing only on digital intake; widen to whole clinic operations and revenue workflows.
Clearwave / patient-access tools	Differentiate through patient-flow visibility, RTM/pre-auth and integrated operations.
Queue/wait-time tools	Position clinIQ as broader than queue management.
EHR / practice-management systems	Complement the system of record rather than proposing a rip-and-replace.

8. Market strategy

Market	Priority	Initial play
United States	P1	Specialty clinics, behavioral health, PT/orthopedics, multi-location groups
United Kingdom	P2	Private specialty/outpatient groups after US motion is validated
UAE/Gulf	P2	Private healthcare groups and digitally progressive clinics
India	P3	Selective multi-site private healthcare; prioritize only with strong local fit

9. Account-selection model

Score every account on ICP fit, active trigger, identifiable pain, access to buying roles and account-expansion potential. Prioritize a small number of high-fit accounts before scaling outbound volume.

Factor	Weight	What good looks like
ICP fit	25%	Strong industry, size and operating fit
Trigger	25%	Visible reason to act now
Pain	20%	Measurable operational or financial problem

Factor	Weight	What good looks like
Access	15%	Reachable buying committee
Expansion	15%	Potential for wider product/services footprint

10. Sales motion

- Build a named account list around specialty, provider count, locations, EHR and growth signals.
- Open with a Clinic Operations & Revenue Assessment, not a generic demo.
- Run workflow discovery and quantify operational pain.
- Demonstrate only the workflows connected to the diagnosed problem.
- Propose a controlled implementation/pilot with measurable success criteria.
- Expand to locations, modules, integration and healthcare engineering.

11. Discovery framework

- How many providers, locations and patient visits are managed?
- Where do patients wait and where does staff time get consumed?
- Which workflow still depends on paper, spreadsheets, calls or manual status checks?
- What is the current EHR and what does it not solve operationally?
- How are prior authorizations tracked?
- Is RTM relevant and, if so, how is eligibility/engagement/billing managed?
- What operational KPI would justify a change?

12. Qualification

- Clear business pain with measurable operational impact.
- Named operational/economic owner.
- EHR/integration environment understood.
- Implementation appetite within a realistic timeframe.
- Ability to define success metrics for a first phase.

13. Demo / workshop / pilot

- Demo the customer's journey, not the product menu.
- Use a current-state / future-state workflow.
- Show patient flow and the strongest revenue/administrative wedge first.
- Define a pilot around one location or workflow where possible.
- Agree measurable outcomes and adoption owners before implementation.

14. Commercial model and business case

- Anchor on operational value, not module count.
- Use public pricing only where current and appropriate; enterprise/integration requirements need validation.
- Quantify staff time, throughput, revenue capture or avoidable administrative burden.
- Model account expansion by locations, modules and integrations.

15. Objection handling

Objection	Response direction
"Our EHR already does this."	Map the specific operational gaps the EHR does not solve and position clinIQ as complementary.
"We cannot disrupt clinic operations."	Use phased workflow adoption and narrow pilot scope.
"Staff will not use another system."	Tie adoption to reducing existing manual steps; involve operations users early.
"We are too small."	Qualify out low-pain practices; focus effort where operational complexity creates ROI.

16. Closing strategy

- Confirm the quantified problem and success criteria before commercial negotiation.
- Map procurement, security, legal and executive approval early.
- Reduce decision risk with a clear first phase, milestones and governance.
- Before signing, define the likely expansion path if the first phase succeeds.

17. Partnerships and channels

- EHR ecosystem partners and healthcare IT consultants.
- Revenue-cycle / RTM advisors where relevant.
- Healthcare associations and specialty communities.
- Implementation/integration partners in target markets.

18. Expansion and cross-sell

- Additional locations and provider groups.
- EHR/API integration and custom healthcare engineering.
- AI/data analytics initiatives.
- Cloud and managed infrastructure where relevant.
- Customer-success reviews to surface adjacent operational pain.

19. 30 / 60 / 90 execution

Period	Focus	Outputs
['0-30 days', 'Validate ICP and messaging', 'Audit existing product customers/pipeline', segment specialty ICPs, build first account list, define assessment offer']	['31-60 days', 'Activate', 'Launch US specialty-clinic account campaigns, run discovery, assessment and tailored demos']	['61-90 days', 'Prove', 'Create qualified pipeline, pilots/proposals, identify highest-converting specialty and refine expansion play']

20. KPIs

- Tier-A accounts researched.
- Executive/operations conversations.
- Assessments completed.
- Demo-to-pilot conversion.
- Pilot-to-paid conversion.
- Average first-year account value.
- Multi-location/module expansion.
- Sales cycle by segment.

21. Relevant commercial experience

- TC+LIMS experience demonstrates workflow discovery and vertical SaaS selling.
- ATL Labs / laboratory digitization shows ability to translate manual workflow into software value.
- Suhana Foods quality-management work demonstrates operational process mapping and product collaboration.

22. Revenue planning principle

Start with bottom-up pipeline math: target accounts → conversations → qualified opportunities → pilots/proposals → wins → expansion. Do not commit to revenue without validating actual pricing, baseline conversion rates and sales capacity.

Planning assumptions are illustrative only and must be validated against The Algorithm's actual pricing, pipeline, customer base, margins and conversion data before being used as forecasts or commitments.